



OFFICIAL

HealthCare to Community Guideline

1. Purpose

To provide information about the HealthCare to Community (HC2C) service at Armadale Health Service (AHS) and how to appropriately refer patients.

2. Applicability

The HC2C team comprises of multidisciplinary staff including nursing and allied health working trans-professionally and specifically review patients who are admitted to hospital with:

- Chronic Obstructive Pulmonary Disease (COPD) and/or
- Heart Failure (HF)

3. Guidelines

The overall aim of the HC2C team is to:

- Improve the patient's experience through patient-centred care
- Coordinate services provided to the patient across the continuum of care
- Improve communication between all health care professionals' patients, family and carers
- Establish stronger relationships with primary health care providers in the community to improve shared care planning

4. Roles and responsibilities

The HC2C team provides support to the patient, their families and/or carers during the transition from hospital to community under the guidance and governance of General Medicine, Cardiology and Respiratory.

The HC2C team provides:

- Screening to identify patients admitted to AHS with COPD and/or HF and have complex care needs and/or at risk of re-presentation or re-admission to hospital
- A multidisciplinary assessment of the person's care needs
- Chronic disease and self-management education to patients with COPD and/or HF
- Strategies to facilitate discharge planning
- Support to clinicians to provide optimise management of HF patients as per the recommendations in the *National Heart Foundation of Australia and Cardiac Society of Australia and New Zealand, Guidelines for the Prevention, Detection and Management of Heart Failure in Australia 2018*
- Information, promotion and completion of referrals to appropriate community services post hospital discharge
- Post-discharge follow-up through the Outpatient Clinics
- Spirometry via the Respiratory Testing Outpatient Clinic

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- Communication and shared care across hospital and primary health care services
- Peer education including nursing, medical and primary health care services

Screening Process

The HC2C Care Coordinators screen all medical inpatients on the first day of hospital admission to the Medical Assessment Unit (MAU), Canning Ward, Colyer Ward, Rehabilitation Ward and Intensive Care Unit (ICU) for COPD and/or HF.

Governance

- HealthCare to Community inpatient activity provides a service under the governance of General Medicine and/or Cardiology.
- The work of the nurse-led HC2C Outpatient clinic is under the governance of General Medicine (GPM408, GPM409) and escalates urgent care of patients (as needed) to the General Medicine Consultant on call (via switch).
- The nurse-led Heart Failure Outpatient Clinics (CAR152, CAR153) works to the governance of Cardiology and escalates urgent care of patients (as needed) to AHS Cardiology and/or the General Medicine Consultant on call (via switch).
- The physiotherapist/spirometry operator-led Respiratory Testing Outpatient Clinic (RES082) works to the governance of AHS and RPH Respiratory and escalates urgent care of patients (as needed) to the General Medicine Consultant on call (via switch).

Patient Eligibility

The Care Coordinators work across AHS, mainly in the General Medicine wards and Outpatient Clinics. This service can be requested as required for patients with COPD and/or HF.

Inclusion Criteria

- Adult patients
- Admitted with an exacerbation or new diagnosis of one/both target chronic diseases (COPD, CHF)
- Reside in the AHS catchment area

Exclusion Criteria

- Patients who primarily reside outside the AHS catchment area
- Patients who have been assessed and planned for permanent residential care
- Patients in palliative care or in end-stage disease management who either have all supports in place or are declining all supports
- Patients declining HealthCare to Community service input

Referrals

INPATIENTS:

Patients with a confirmed presenting complaint/diagnosis of COPD via:

- e-referral: Create Referral/inpatient/General Medicine/HealthCare to Community;
- direct referral to a Care Coordinator via phone number (0408 957 022 or 9391 2732) or MDT;

Patients with a confirmed presenting complaint/diagnosis of HF via:

- e-referral: Inpatient/General Medicine/HealthCare to Community
- direct referral to the Heart Failure CNS via 0439 321 316 or 0434 601 845.

OUTPATIENTS:

Patients requiring outpatient support following an admission with COPD via:

- e-referral: Outpatient/General Medicine/HealthCare to Community

Patients with a confirmed diagnosis of HF via:

- e-referral: Outpatient/Cardiology/Heart Failure Nurse

5. Legislative context

- HDWA
 - Clinical Handover Policy MP 0095
 - Consent to Treatment Policy OD0657/16
- EMHS
 - Patient Identification and Procedure Matching Policy
 - Medication Management
 - Clinical Handover Procedure

6. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards:

- Standard 2 – Partnering with Consumers
- Standard 6 – Communicating for Safety

7. References

Chronic Obstructive Pulmonary Disease. Australian Institute of Health and Welfare. 2024

Consensus Statement on the Current Pharmacological Prevention and Management of Heart Failure. Medical Journal of Australia. 2022.

National Heart Foundation of Australia and Cardiac Society of Australia and New Zealand, 2018, Guidelines for the Prevention, Detection, and Management of Heart Failure in Australia 2018. National Heart Foundation of Australia.

Sustainable Health Review, 2019, Sustainable Health Review: Final Report to the Western Australian Government. Department of Health, Western Australia.

State of Precision Medicine for Heart Failure with Preserved Ejection Fraction in New Therapeutic Age. ESC Heart Failure. 2025

Thoracic Society of Australia and New Zealand and Lung Foundation Australia, 2024, COPD-X Plan: Australian and New Zealand Guidelines for the Management of Chronic Obstructive Pulmonary Disease 2021. Version 2.77. Lung Foundation Australia.

8. Glossary

The following definitions are relevant to this policy.

Term	Definition
Complex Care Needs	Patients with substantial and ongoing healthcare needs that require a coordinated approach across multiple disciplines. The care of these patients is often outside the standard model of care.
COPD	Chronic obstructive pulmonary disease (COPD) is a preventable and treatable lung disease characterised by chronic obstruction of lung airflow that interferes with normal breathing and is not fully reversible.
Heart Failure	Heart failure is a complex clinical syndrome with typical symptoms and signs that generally occur on exertion, but can also occur at rest (particularly when recumbent). It is secondary to an abnormality of cardiac structure or function that impairs the ability of the heart to fill with blood at normal pressure or eject blood sufficient to fulfil the needs of the metabolising organs.

9. Document owner

Director Allied Health and Ambulatory Care

Enquiries relating to this guideline may be directed to:

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10. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	10 12 2025	10 12 2028	Full review, minor updates throughout document.

11. Authorisation

This guideline has been authorised and issued by:

Authorised by	Danielle Kilmurray - Director Allied Health & Ambulatory Care
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